

1 · FRAMEWORK

Every tachycardia = **increased automaticity** (node/ectopic focus) or **re-entry circuit** (re-entry is faster). Then sort with two questions: **regular vs irregular** and **the P waves**.

Stable, unsure? Slow AV conduction (vagal or adenosine) to expose hidden atrial activity — saw-tooth, retro-P, or organized atrial tach.

2 · THE SEVEN SVTs

Rhythm	ECG	Mgmt
Sinus tach	Reg; nl sinus P	Treat driver
Atrial tach	Reg; non-sinus/inv P	AVN blocker + driver
AVNRT	Reg; no sinus P (retro)	Vagal / adenosine
AVRT orthodromic	Reg narrow; retro-P, short R-P	Vagal / adenosine
AVRT antidromic	Reg wide ; delta + short PR	Procainamide / DCCV
Atrial flutter	Saw-tooth, atrial ~300, 2:1	Rate control
Atrial fib	Irreg-irreg; no P	Rate control + AC
MAT	Irreg; ≥3 P morphologies	Treat cause (COPD)

(Board lists "7 SVTs"; AVRT shown as orthodromic + antidromic of one re-entry mechanism.)

3 · THREE MANAGEMENT BUCKETS

- **Address the driver:** sinus tach, atrial tach, MAT
- **Interrupt re-entry** (vagal / adenosine): AVNRT, AVRT
- **Rate control** (AV-nodal blockers): AF, atrial flutter
- **Unstable at any point** → synchronized cardioversion

4 · ADENOSINE & VAGAL

- **Vagal first:** modified (REVERT) Valsalva — strain into 10-mL syringe, then supine + passive leg raise (~43% vs ~17%).
- **Adenosine** 6 → 12 → 12 mg rapid push + flush: terminates AVNRT/AVRT, **unmasks** flutter/atrial tach, **no effect on VT** — diagnostic + therapeutic.

SAFETY / AVOID

Pre-excited AF (WPW): irregular + **wide** + very fast.
AVOID AV-nodal blockers (adenosine, CCB, β-blocker, digoxin) → can accelerate accessory-pathway conduction → VF. Give **procainamide** or **synchronized cardioversion**.

MAT: avoid non-selective β-blockers with active bronchospasm; correct K/Mg.

VT clue: wide-complex with extreme right-axis (positive aVR) favors VT — when in doubt, treat as VT.